



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : شيماء سعد عبدالسلام دويدار
المحافظة : المنوفية الحي / المدينة : اشمون
رقم البطاقة : 28702021701301 رقم التليفون : 01125983514
العنوان : سسسسسسسسسس
العمر (تاريخ الميلاد) : 02-02-1987 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 06-10-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: ----- التوقيع: -----
الاسم: د/ندى حسن الوظيفة: ----- التوقيع: -----
الاسم: د/نرمين رمضان الوظيفة: ----- التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة الأبحاث والمرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: شيماء سعد عبدالسلام دويدار
National ID: 28702021701301



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

شيماء
محمد عبد السلام نويد
١٩ ش ناصر بوجه شطوط
مركز اشمون - المنوفية

١٩٨٧/٠٢/٢٢ ٢٨٧.٠٢.٢١٧.٠١٣.٠١

K10678225

DMS
٥٧٩٦

٢٠٢٣/١٢ ٢٨٧.٠٢.٢١٧.٠١٣.٠١

دبلوم تجارة
أنثى
صلاح محمد محمود
البطاقة سارية حتى ٢٠٢٥/١٢/٢٤

ملزوجة

١٢

05/04/2026 (8)

MDT FORM

Name:	Shimaa Saad Abdelkalam Dawdar	MRN:	2911
Age:	39 Y	ID:	28782021701301
Menopausal Status:	chemotherapy inducing amenorrhea	Phone:	1277792586
NRS SCORE:		Marital Status:	Married
BMI:		Family History:	
CO Morbidity:	ANDRAL FOR MR	Reason Of MDT:	

Clinical Data:

R

EX:	IDC
ER:	3/8
PR:	4/8
HER2:	AMPLIFIED Ki67: 15%

Sonomammography:

Mets Workup:

Free

History:

PATIENT RECEIVED 6 CYCLES TOMP ENDED ON 19/2/2026 and underwent rt sm

pop:

no residual viable tumor

SLNB: 0/4

YP10, YP40

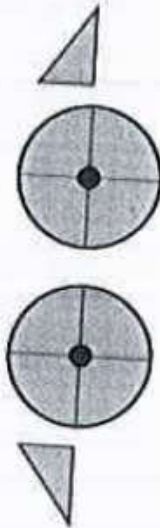
Recommendations:

MDT Outcome & Plan

continue pheno for one year

Tam as Chemoprevention for 5 years

L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. محمد مصطفى	د. محمد الفاروق				

Name: <u>شيماء سعد عبدالسلام دويدار</u>		
MR. : <u>2911</u>		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نتشرف بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمناوبة وجد انها		
RT breast cancer ER 3/8, PR 4/8, HER2 3+ (amplified). The patient underwent multidisciplinary treatment with TCHP, receiving 4 cycles of TC followed by 3 cycles of PHESGO. For cycle 5, the regimen was TC + 4 cycles of PHESGO, with taxotere 130 mg, carboplatin AUC 5, and phesgo 600 sc. An echocardiogram on 26/01/2026 showed an ejection fraction of 68%. The patient's diagnosis was confirmed as multicentric breast cancer, CT2 N0 M0, ER 3/8, PR 4/8, HER2 SISH amplified (SA 1.76). For cycle 5, the plan was taxotere 130 mg, carboplatin AUC 5, and phesgo 4 cycles, with an ejection fraction of 68% on 26/01/2026. During neoadjuvant chemotherapy, the patient received TCHP with SA 1.76. For cycle 6, the regimen was taxotere 130 mg, carboplatin AUC 5, and phesgo 5 cycles, with an ejection fraction of 68% on 26/01/2026. The patient also received cycles 6 and 7 of phesgo and additional pharmacologic support. Clinical pharmacy review noted the patient was on cycle 6 of phesgo with an ejection fraction of 68% on 26/01/2026. Pharmacy review documented the patient was on cycle 7 of phesgo plus zoladex 10.8 mg, with an ejection fraction of 68% on 26/01/2026. The patient underwent surgery with SM + ALND, YOT0 YPNO, and was started on adjuvant tamoxifen and phesgo. For cycle 8, the patient received phesgo plus tamoxifen, supportive therapy, and zoladex 10 / 04, with an ejection fraction of 64%. For cycle 10, the patient was scheduled to receive phesgo (third on the same echo), with a new echocardiogram planned for the next visit, and continued tamoxifen and supportive therapy. The patient has completed 10 cycles of phesgo to date.		
تحريراً في ,		
	الوقت :	التاريخ : 21-09-2026
هنا أسامه عبد الرحيم عبد البر		الطبيب المعالج :
يعتمد ,		

2911

Patient Name : الأستاذ / شيماء سعد عبدالسلام
Patient No. : 28702021701301
Path No. : 973/26
Sex : Female
Age : 39 Y
Referring Doc :

Request No. : 13005820
Request Date : 11/04/2026
Verification Date : 21/04/2026
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer (Post-NACT).

Nature of specimen: Right simple mastectomy & Sentinel lymph nodes biopsy.

Gross Examination:

* Right simple mastectomy specimen measured 16x10x5 cm, with covering skin ellipse 16x8 cm. Nipple & areola looked unremarkable grossly.
* Serialling showed a tumor bed site measured 6x5 cm, located 1.0 cm from the least deep margin.

* Sentinel lymph nodes: Multiple fibrofatty tissue pieces collectively measured 4x3cm, entangling 4 lymph nodes, the largest measured 1x2x0.5 cm.

* 9 Blocks:

1:6- TBS., 7- Nipple., 8- Deep margin., 9- SLNs.

- Gross was done by; Dr. Heba El Shafeey.

** Frozen (Intra-operative) results:

- SLNs: Three free sentinel lymph nodes (0/3).

Microscopic Examination:

* Histological type: No residual viable remnants of the previously diagnosed invasive tumor.

* Angioinvasion: Absent.

* Ductal carcinoma in situ: Absent.

* Nipple & areola: Free.

* Deep margin: Free.

* Sentinel lymph nodes: Four lymph nodes free of metastatic deposits (0/4).

* Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:

Right breast cancer (Post-NACT), Right simple mastectomy & Sentinel lymph nodes biopsy:

Verified by :

Dr. Heba Hasan

Printing Date : 21/04/2026 11:24:01

Page 1 of 2

Printing By : marwa mohamed youss



Patient Name : الأستاذ / شيماء سعد عبدالسلام
Patient No. : 28702021701301
Path No. : 973/26
Sex : Female
Age : 39 Y
Referreing Doc :

Request No. : 13005820
Request Date : 11/04/2026
Verification Date : 21/04/2026
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

- No residual viable remnants of the previously diagnosed invasive tumor.
- Ductal carcinoma in situ: Absent.
- Sentinel lymph nodes: Four lymph nodes free of metastatic deposits (0/4).
- Nipple & areola: Free.
- Deep margin: Free.
- Pathological complete response to therapy.
- Stage: ypT0, ypN0.

Signature
Dr.Walaa Ibrahim Ghoneim

Verified by :
Dr.Heba Hasan



Patient Name: (20702021701501)
Patient No.: 151313/25
Sex: Female
Age: 38 Y
Referring Doc:

Regional No: 13000865
Request Date: 22/09/2026
Verification Date: 01/10/2026
Financial Category: (1) - (2) - (3)
Department: Out Patient
Registered By: Mrs. Mayada Hagazy

Pathology Report

- PR: Positive (score 4/8).
- HER2: Equivocal (score 2+).
- SIS-1: Amplified with clusters.
- Ki67: 15%.
- Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010: Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

• Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013: IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faintly perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faintly perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature:
Dr. Walaa Ibrahim Ghoneim
Dr. Hiba El-shafeey

Verified by:
Dr. Hiba Hasan

Printing By: Mrs. Mayada Hagazy

Printing Date: 04/10/2025 09:05:40

Page 2 of 3



Room No. (A) - First Floor, Building No. 987, Nile Canal - El-Dokki, Cairo, Egypt
مركز سرطان النيل - مبنى رقم (أ) - طابق الأول - 987 - قناة النيل - الدوكي - القاهرة - مصر

Patient Name: Shaiman Saad Abd Al Salam Dewidar	Date: 02/10/2025
Exam: CT CAP+C (CT Chest Abd. Pelvis)	Age: 38 Year(s)
Medical ID: 2687/2025	Pat. ID: 121866

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

****A known case of right breast cancer**

I Chest: -

- Right breast mass 24 x 21 mm
- Multiple enlarged right axillary L. Ns , the largest measures 15 x10 mm with preserved fatty hilum.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes .
- Tiny subpleural pulmonary calcified nodule 3mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is mildly enlarged in size with homogenous CT texture. No focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

III The scanned skeleton:

- No worrisome lytic or sclerotic bony lesions.

Patient Name: Shaimaa Saad Abd Al Salam Dewidar

Date : 02/10/2025

Exam : CT CAP+C (CT Chest Abd. Pelvis)

Age : 38 Year(s)

Medical ID: 2687/2025

Pat. ID: 121866

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND

PELVIS: -

****A known case of right breast cancer**

I Chest: -

- Right breast mass 24 x 21 mm
- Multiple enlarged right axillary L. Ns , the largest measures 15 x10 mm with preserved fatty hilum.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitory lesions.
- No hilar or mediastinal lymph nodes .
- Tiny subpleural pulmonary calcified nodule 3mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is mildly enlarged in size with homogenous CT texture. No focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

III The scanned skeleton:

- No worrisome lytic or sclerotic bony lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

Opinion:

No hepatic , pulmonary or bony metastatic deposits

Dr. Reham Khedr, MD

Name : Shaimaa Saad Abdelsalam Doudar
Branch : Dokki
Ref.Physician Prof : Gostaf Roseh Hospital

Date : 20-10-2025 11:38
Medical No. : 229452
Ins.Company : تعاقد مستشفيات

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 38-year-old lady, with history of right breast cancer, referred for follow up.

Technique:

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings:

- No any abnormal-active or cold focal bony lesions that may account for metastatic bone disease.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Patient Name: Shaimaa Saad Abd Al Salam	Date: 02/10/2025
Dewidar	Age: 38 Year(s)
Exam: MAMMOGRAPHY BOTH	Pat ID: 121866
Medical ID:	

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 37-year-old female with negative family history with pathologically proven right IDC.

Findings:

Right breast:

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- UOQ & UIQ, through-out the whole breast thickness, global asymmetry with overlying suspicious segmental coarse heterogenous microcalcifications over an area measuring about 8.5 x 8 x 7 cm with surrounding distortion, these changes reaching the nipple.
- Ultrasound demonstrated UOQ & UIQ, 9-2 o'clock, through-out the whole thickness of the breast altered parenchyma with prominent ducts reaching 3 mm in caliber with soft tissue echogenicity, microcalcifications and internal vascularity within.
- Those changes are seen reaching the nipple.
- Two level 1 indeterminate axillary LNs are noted showing diffuse cortical thickness reaching 3.5 mm & 4 mm.

Left breast:

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses asymmetry, distortion or suspicious microcalcifications.
- Ultrasound demonstrated heterogeneous breast parenchyma with mottled appearance and multiple simple small cysts yet no solid lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 6): Upper breast altered parenchyma with microcalcifications and ipsilateral level 1 two indeterminate lymph nodes.

Left breast (BIRADS 2): breast adenosis with cyst formation.

Recommendation:-

For CESM:

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations

Suspected Cancer Risk

BIRADS 0: Non-informative study.	0%
BIRADS 1: Normal study, routine screening.	0%
BIRADS 2: Benign findings, routine screening.	2%
BIRADS 3: Probably benign, for close follow up.	50%
BIRADS 4: Probably malignant, for biopsy.	

Patient Name : الأستاذة / ميادة هجazy
Patient No. : 28702021701301
Path No. : SR-1313/25
Sex : Female
Age : 38 Y
Referring Doc :

Request No. : 13000865
Request Date : 22/09/2025
Verification Date : 01/10/2025
Financial Category : Out Patient
Department :
Registered by : Mrs. Mayada Hegazy

Pathology Report

CLINICAL DATA
Right breast mass

NATURE OF SPECIMEN
Block and slide revision

GROSS EXAMINATION

One block labelled by (2025) & one slide were received.

MICROSCOPIC EXAMINATION

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade 2
- * DCIS: Positive
- * Angiolysis: Absent
- * Perineural invasion: Absent
- * IPT (Positive control showed appropriate reactivity):
- * ER: Weak nuclear reaction in 2 % tumor cells (Allred score 3/8)
- * PR: Weak nuclear reaction in 30 % tumor cells (Allred score 4/8)
- * HER2: Moderate complete membrane reaction in >10 % of tumor cells (score 2+)
- * SISH: amplified with clusters
- * Ki 67: 15 %

DIAGNOSIS

Right breast mass, block and slide revision

Invasive duct carcinoma, Grade 2 with positive duct carcinoma insitu, cribriform, solid and comedo types, intermediate grade

IPT:

- * ER: Positive (score 3/8)

Verified by:
Dr. Heba Hasan

Printing By: Mrs. Mayada Hegazy

Printing Date: 04/10/2025 09:05:46

Page 1 of 3

نوفة رقم (4) - الدور الأول - مبنى رقم (987) - مستشفى النور - قصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987 - Nile Corniche - Mistr El Qadima - Cairo, Egypt

